

Chapter III

Structural Eugenics in Contemporary Alberta

Six Mechanisms in the Contemporary Architecture

Chapter II established the analytic framework: eugenics is broader than surgical sterilization; “structural eugenics” is the documented disability-studies term for policies producing eugenic effects without requiring eugenic intent; and consultation under CRPD Article 4(3) is a binding international standard that Alberta has not met in the development of contemporary disability policy. This chapter applies that framework to six specific Alberta policies currently in force or scheduled to come into force in 2026. The inventory is not exhaustive. The six mechanisms documented here are those for which the primary-source record is the most complete and the structural-eugenics mapping is the most direct.

Each section identifies the policy, the documented effect, and the historical eugenic mechanism it parallels. The analytic claim is the effect claim defined in Chapter II: that these policies produce measurable reductions in disabled people’s capacity to form households, to reproduce, to participate in community, to maintain long-term partnerships, and to access the material conditions of a livable life. The intent claim is not the claim made here. The effect claim is what the documented record supports.

The Cohabitation Rule

Under current AISH policy, the income of any adult sharing a household with an AISH recipient may be assessed as part of the household income calculation and used to reduce the recipient’s benefit. The rule applies regardless of the nature of the relationship: a married spouse, a common-law partner, a roommate sharing rent, an adult child temporarily living at home, an elderly parent in a granny suite, and a co-parenting partner who is not a romantic partner are all treated under the same household-income provision. The rule contains narrow exemptions, notably for paid live-in caregivers under documented arrangements; the exemptions do not cover practical roommate, co-parenting, or multi-generational household arrangements common in low-income, Indigenous, and immigrant communities.¹

On May 12, 2026, the Government of Alberta filed AR 96/2026, a new General Regulation governing both AISH and ADAP, in force on July 2, 2026. Section 8(4) of the regulation introduces what the government’s fact sheet describes as an “allowance for two-client couples living together”: where two adults in the same household each receive disability income assistance, each partner receives 88 percent of the maximum individual benefit. The reduction applies regardless of which program each partner is on.²

The mathematics, drawn from the government's own base rates, are as follows. Two AISH recipients sharing a household lose \$5,587.20 per year against the baseline of two unreduced AISH benefits. A mixed AISH-and-ADAP household loses \$7,699.20 per year. Two ADAP recipients lose \$9,811.20 per year. These figures are calculated before any employment income is considered and before the federal Canada Disability Benefit clawback discussed in section 3 of this chapter, which compounds against the same household.³

The structural effect is documented and direct. Partnership between disabled adults receiving AISH or ADAP is, after July 2, 2026, a \$5,587 to \$9,811 annual penalty against the household. Practical household-sharing arrangements between an AISH recipient and any other adult — romantic, familial, platonic — place the recipient's benefit at risk of reduction through the broader cohabitation rule. The recipient is, in the language of Inclusion Alberta's ADAP fact sheet, "structurally incentivized to remain alone."⁴

The historical eugenic mechanism the rule parallels is direct. The marriage prohibitions, immigration exclusions, cohabitation restrictions in residential institutions, and active provincial discouragement of family formation by disabled people documented in Chapter I operated through legal prohibition. The contemporary mechanism operates through financial penalty. The legal prohibition has been repealed; the financial mechanism that produces equivalent isolation has not been.

Sub-Survival Income: AISH, ADAP, and the Housing Crisis

AISH provides a maximum living allowance of \$1,940 per month in 2026. ADAP, the program to which all AISH recipients without severe and profound developmental disabilities, PDD eligibility, or palliative/terminal conditions will be transitioned on July 1, 2026, provides a maximum of \$1,740 per month. Recipients who are transitioned receive a transition payment maintaining the AISH rate until December 31, 2027; on January 1, 2028, the payment drops to the ADAP rate.⁵

Average one-bedroom rent in Edmonton and Calgary ranged from \$1,500 to \$1,900 in 2024, with documented year-over-year increases of 17.5 percent province-wide and up to 21.3 percent in three-bedroom markets, against a provincial average of \$2,138 per month for three-bedroom units. Alberta does not impose a residential rent-increase cap.⁶

The arithmetic produces the documented condition. A single AISH recipient on the maximum living allowance at the lower end of one-bedroom rent has, after rent, between forty and four hundred forty dollars per month for all other costs — food, utilities, transportation, medical out-of-pocket expenses, clothing, household supplies. Statistics Canada and Canada Food Price Report estimates for single-adult grocery costs in 2026 are eight hundred to one thousand dollars per month. Alberta utility estimates are three hundred eighty-five to five hundred ten dollars per month. The single recipient on private-market rent is operating, by these calculations, at a structural deficit of seven hundred fifty dollars or more per month.⁷

In Alberta's 2023–2024 fiscal year, 83.6 percent of AISH recipients had no employment income. The Government of Alberta's framing of ADAP as an empowerment program enabling Albertans with

disabilities to pursue employment operates against a documented baseline in which the great majority of recipients are not, on the available evidence, in a position to do so.⁸

The historical eugenic mechanism the income structure parallels is institutional segregation. The historical residential institutions documented in Chapter I — the Provincial Training School at Red Deer and the network of mental hospitals and guidance clinics — excluded disabled people from full community participation by removing them from the community. The contemporary income structure excludes disabled people from full community participation by setting income at a level that does not permit the housing, food, transportation, or social engagement that participation requires. The mechanism has changed from institutional confinement to economic confinement. The effect — exclusion from the conditions of community life — is the same.

The Canada Disability Benefit Clawback

The federal *Canada Disability Benefit Act* was passed in 2023 as a poverty-reduction measure for working-age Canadians with disabilities. The benefit pays up to \$200 per month, subject to a Disability Tax Credit prerequisite and an income-tested eligibility framework. The federal government, in introducing the benefit, explicitly requested that provinces and territories not reduce their own disability income payments when recipients also receive the federal benefit.⁹

In a letter to AISH recipients dated July 2, 2025, the Government of Alberta notified recipients that the Canada Disability Benefit would be treated as non-exempt income under AISH and ADAP. Recipients who did not demonstrate that they had applied for the CDB would see an equivalent \$200 deduction from their provincial benefits. Alberta is the only province or territory in Canada to claw back the Canada Disability Benefit dollar for dollar. Every other province and territory fully exempts the federal payment.¹⁰

The fiscal effect is direct. Applied to approximately 79,290 AISH recipients at \$200 per month, the clawback captures approximately \$190 million per year in federal transfer payments that would, in any other Canadian province, be received by disabled recipients. In Alberta, the federal investment is redirected to the provincial budget; the net benefit to the disabled recipient is zero.¹¹

The structural effect is to nullify federal poverty-reduction policy at the provincial level for the recipient population the federal policy was designed to serve. The historical eugenic mechanism the clawback parallels is the active provincial discouragement of disabled people from accessing resources available to non-disabled people. The historical version operated through institutional segregation. The contemporary version operates through the structural capture of federal transfer payments designed to lift disabled Albertans above the poverty line.

The FSCD Three-Year Wait

The Family Support for Children with Disabilities program funds respite, in-home support, therapy services, and equipment for Alberta children under eighteen with severe disabilities. It is the primary provincial mechanism through which families of disabled children access intervention services during the developmental years.¹²

In October and November 2024, Inclusion Alberta surveyed 540 families from 79 of Alberta's 87 electoral districts about their FSCD experience. The findings, published in January 2025, document an average wait of three years from application to full range of FSCD services. Eighty-five percent of families who had received the partial Family Support Services agreement that follows initial eligibility were still without a caseworker for the full Child Focused Services component. Fifty-eight percent of families reported negative effects from going without supports, including job loss, missed developmental windows, and measurable physical and mental health deterioration.¹³

The Auditor General of Alberta examined FSCD in 2022 and reported that caseworker training and oversight processes were inadequate and that families “may not receive the supports and services they qualify for.” The Government of Alberta received those findings in 2022. Caseworker caseloads have since been documented at four hundred families per worker against a standard reference of one hundred. Seven temporary caseworker contracts were not renewed in September 2024. The FSCD budget increase in Budget 2025 was 3.61 percent against a documented population-plus-inflation requirement of 7.3 percent — a real-terms reduction of approximately 3.69 percentage points.¹⁴

The structural effect is documented in the developmental literature on early intervention. The years between roughly two and five years of age constitute the period in which intervention services for developmental disability produce the largest measurable gains in functional outcomes. A three-year wait from application moves children past this window before services begin. A child diagnosed at four whose family applies the same day will not, on the documented average, receive full FSCD services until age seven — outside the window the program's own evidence base identifies as most consequential.¹⁵

The historical eugenic mechanism the FSCD wait parallels is the institutional intervention in disabled children's developmental trajectory documented in Chapter I. The historical version operated through institutionalization at the Provincial Training School and similar facilities, where children were separated from family and community on the asserted ground that institutional care was protective. The contemporary version operates through administrative wait time. The mechanism has changed from confinement to delay. The effect — the developmental trajectory the disabled child would otherwise have had is not the trajectory they receive — is the same.

PDD: Guardianship, Trusteeship, and the Unreported Waitlist

The Persons with Developmental Disabilities program supports adults eighteen and over with developmental disabilities of onset before age eighteen. It funds residential support, employment support, community access, and individualized care planning. PDD currently supports more than thirteen thousand adults in Alberta. The waitlist — the number of adults who are eligible for PDD but not receiving services — has not been publicly reported by the Government of Alberta in three years. The last published waitlist data are from 2021.¹⁶

Inclusion Alberta surveyed 540 PDD-connected individuals in late 2024 specifically because the Government would not release waitlist data. The findings, published in January 2025, document that seventy percent of respondents still need to meet with a caseworker to develop the outcome plan that determines their services. Forty-one percent of those who do receive supports report that the supports are

not helping. The PDD budget increase in Budget 2025 was 5.94 percent against the 7.3 percent population-plus-inflation requirement — a real-terms reduction of approximately 1.36 percentage points.¹⁷

The structural framework PDD operates within is the *Alberta Adult Guardianship and Trusteeship Act*. The Act establishes a substitute-decision-making framework for adults whose capacity to make personal and financial decisions is operationally impaired. The Act's framework conflicts with Article 12 (equal recognition before the law) of the CRPD, which the UN Committee on the Rights of Persons with Disabilities has identified, in General Comment No. 1, as requiring transition from substitute decision-making to supported decision-making across signatory jurisdictions.¹⁸

The historical eugenic mechanism the PDD framework parallels is the assertion of institutional authority over the lives of disabled adults. The Alberta Eugenics Board documented in Chapter I asserted authority to determine that disabled adults' reproductive lives were an appropriate site for state decision-making. The contemporary AGTA framework asserts authority over disabled adults' personal-and-financial decisions through a substitute-decision-making model that international human rights law has identified as requiring reform. The mechanism has changed from sterilization authority to substitute-decision-making authority. The operating premise — that disabled adults are appropriate subjects of external decision-making in ways non-disabled adults are not — is the operating premise of both periods.

Institutional Care: The Separation of Couples

The Alberta continuing care system, encompassing long-term care facilities (Continuing Care Home Type A) and designated supportive living facilities (Continuing Care Home Type B), operates with documented bed-availability constraints, facility-specialization patterns that distribute different levels of care across distinct facilities, and a fee-determination architecture based on couple's combined-income calculations. The combination produces a documented pattern in which spouses are forced to live apart when one requires institutional care that the other does not.¹⁹

In June 2019, CBC News reported the case of Martha and Willard Farnell of Calgary, Alberta. The Farnells had been married for sixty-six years. Following Willard's dementia diagnosis and admission to a long-term care facility approximately twenty minutes' drive from Martha's residence, the Alberta continuing care system did not provide a mechanism through which the couple could continue to share accommodation. The CBC report documented Sweden's policy position treating forced spousal separation in long-term care as inhumane, and the Netherlands "Living Apart Together" suites model that permits couples to live in separate but connected rooms or apartments within the same care facility.²⁰

The peer-reviewed medical literature documents that severe loneliness affects up to forty-two percent of long-term-care residents, against ten percent prevalence in older adults living in the community. The mortality and morbidity consequences of social isolation in this population are documented at scale.²¹

Ontario's long-term care regulatory framework, since January 1, 2018, has required every long-term care home in the province to have at least two beds available for spouses. Alberta has no equivalent requirement. The provincial Government's own acknowledged capacity need is ten thousand new continuing-care spaces over three years; documented commitments are at a small fraction of that figure.²²

The historical eugenic mechanism the institutional-care separation pattern parallels is direct. The cohabitation restrictions enforced in the historical residential institutions documented in Chapter I — the Provincial Training School, the network of mental hospitals — separated disabled adults from spouses, family, and community as a matter of institutional policy. The contemporary version separates senior spouses, one or both of whom is disabled, through bed-allocation and facility-specialization mechanisms that the policy architecture characterizes as administrative necessity. The mechanism has changed from policy prohibition to administrative allocation. The effect — partnered disabled adults are separated from their partners by the institutional system at the moment they most need continued partnership — is the same.

What This Chapter Establishes

The six mechanisms inventoried in this chapter are not the complete inventory of structural eugenics in contemporary Alberta. They are the inventory for which the primary-source record is the most complete. Each is documented in current Alberta legislation, regulation, or program policy. Each is verified against current government data or against survey work conducted by Inclusion Alberta, the Auditor General of Alberta, or the peer-reviewed medical literature. None of the six requires speculative inference; each operates as documented.

Three observations follow.

First, the six mechanisms operate together. The cohabitation rule restricts partnership. Sub-survival income restricts community participation. The CDB clawback nullifies federal poverty-reduction intervention. The FSCD wait restricts developmental trajectory. PDD guardianship restricts adult autonomy. Institutional care separation restricts continued partnership in aging. Taken together, they constitute an architecture in which disabled people are structurally separated from the relational, developmental, financial, and social conditions of full life across the lifecycle. The architecture is not the product of a single decision. It is the cumulative product of decisions taken over decades, each defensible in isolation, none requiring eugenic intent.

Second, the historical mechanisms documented in Chapter I have direct parallels in this inventory. The marriage prohibitions of the 1928–1972 period are paralleled by the cohabitation rule. The institutional segregation of the Provincial Training School period is paralleled by the income and housing structure that prevents community-based independent living. The institutional cohabitation restrictions are paralleled by the long-term-care separation pattern. The institutional authority over disabled adults’ lives asserted by the Eugenics Board is paralleled by the Adult Guardianship and Trusteeship Act substitute-decision-making framework. The developmental intervention of the historical institutionalization period is paralleled by the FSCD wait. The active discouragement of disabled family formation documented in the 1928–1972 record is paralleled by the integrated effect of all six contemporary mechanisms.

Third, none of the six mechanisms documented here was developed through a consultation process meeting the standard CRPD Article 4(3) requires. The cohabitation rule was filed by regulation on May 12, 2026, ten days after the Inclusion Alberta media release identifying Bill 12 as eroding disability rights, and was not preceded by consultation with disability community representative organizations on its

specific 88-percent mechanism. The CDB clawback was communicated to recipients by letter on July 2, 2025, without consultation. The Bill 12 framework producing the AISH-to-ADAP transition was passed on December 11, 2025 over the documented opposition of every major Alberta disability advocacy organization. The FSCD, PDD, and institutional-care frameworks operate within budget and policy cycles that do not, on the available record, include the consultation through representative organizations the Convention requires.

The mechanism has changed. The architecture has not.

Notes — Chapter III

1. Government of Alberta, Assured Income for the Severely Handicapped Regulation, prior framework at Alta Reg 91/2007 (repealed and replaced May 2026); The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, sections on the cohabitation rule and the isolation trap. The narrow exemption for paid live-in caregivers does not extend to practical roommate, co-parenting, or multi-generational household arrangements.
2. Assured Income for the Severely Handicapped General Regulation, AR 96/2026, filed May 12, 2026, in force July 2, 2026, section 8(4); Government of Alberta, Assisted Living and Social Services, “Alberta Disability Assistance Program Fact Sheet,” May 12, 2026.
3. Calculations based on the Government of Alberta’s published AISH base rate (\$1,940/month) and ADAP base rate (\$1,740/month) for 2026; see The Alberta Disability System Breakdown, *Income Crossover Analysis*, revised May 2026, for full worked calculations of dual-recipient household income losses under AR 96/2026 section 8(4).
4. Inclusion Alberta, *ADAP Fact Sheet*, December 2025; see also The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, section on the isolation trap.
5. Government of Alberta, AISH and ADAP program documentation, 2025–2026; The Alberta Disability System Breakdown, *Income Crossover Analysis*, revised May 2026, on the transition payment expiring December 31, 2027.
6. Canada Mortgage and Housing Corporation, Rental Market Report 2024; Urbanation/Rentals.ca, National Rent Report, June 2024 (17.5 percent Alberta year-over-year increase); GIL Property Management, “Alberta Rent Prices in 2024,” November 2024 (three-bedroom average \$2,138/month, 21.3 percent year-over-year increase). Alberta does not impose a residential rent-increase cap; British Columbia, by comparison, set its 2026 limit at 2.3 percent.
7. Dalhousie University, *Canada’s Food Price Report 2026*, December 2025; Alberta utility cost estimates compiled in The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, sections on the math of housing affordability.
8. Government of Alberta, AISH program data, 2023–2024 fiscal year; Inclusion Alberta, *ADAP Fact Sheet*, December 2025.
9. *Canada Disability Benefit Act*, SC 2023, c 9; Employment and Social Development Canada, Canada Disability Benefit program documentation, 2025.
10. Government of Alberta letter to AISH recipients, July 2, 2025 (notification of CDB treatment as non-exempt income); CBC News, “Federal disability benefit to be deducted from AISH payments,” Bellefontaine, M., February 2025; Alberta Doctors’ Digest, “Pushing back against changes that make life harder for disabled Albertans,” November/December 2025; Gillian Petit, “The Canada Disability Benefit and Provincial Clawback: A Fiscal Transfer Analysis,” *Canadian Tax Observatory*, April 22, 2026.
11. Calculations based on Government of Alberta AISH Caseload Open Data (approximately 79,290 recipients as of September 2025) and the \$200/month maximum CDB payment.
12. Government of Alberta, Family Support for Children with Disabilities program documentation, alberta.ca.
13. Inclusion Alberta, *Experiences with Family Support for Children with Disabilities* (FSCD Survey Report), January 2025. 540 respondents from 79 of 87 Alberta electoral districts.
14. Auditor General of Alberta, *Report on FSCD*, 2022 (caseworker training and oversight findings); Hold My Hand Alberta / The Albertan News, “Some Alberta caseworkers that help disabled children have 100s of clients,” September 16, 2024; Government of Alberta, Budget 2025; Inclusion Alberta, Budget 2025 analysis.
15. The developmental early-intervention window for children between approximately ages two and five is documented across the developmental disability literature. See, e.g., discussions in the FSCD Survey Report (Inclusion Alberta, January 2025) and the broader developmental pediatrics literature on intervention timing.

16. Inclusion Alberta / ALIGN, *PDD Current State: Critical and Urgent: Experiences with Persons with Developmental Disabilities*, January 2025; CBC News, “Provincial support delays are hurting disabled Albertans and families, reports find,” Bellefontaine, M., January 23, 2025.
17. Inclusion Alberta / ALIGN, *PDD Current State*; Government of Alberta, Budget 2025; Inclusion Alberta, Budget 2025 analysis.
18. *Adult Guardianship and Trusteeship Act*, Statutes of Alberta, 2008 (provincial framework for substitute decision-making and trusteeship arrangements); Committee on the Rights of Persons with Disabilities, General Comment No. 1 (2014) on Article 12 of the CRPD (equal recognition before the law), document CRPD/C/GC/1.
19. Government of Alberta, continuing-care framework, including *Continuing Care Act* and related regulations; The Alberta Disability System Breakdown, *Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act*, Part One, Section 1.10.
20. Melissa Mancini, “Separating Couples in Long-Term Care: Some Nations Call It Inhumane, in Most of Canada It’s Routine,” CBC News, June 2, 2019, <https://www.cbc.ca/news/health/national-long-term-care-couples-housing-1.5159102>. The CBC report documents the Farnell case and comparator-jurisdiction approaches.
21. Joanna Briggs Institute, documentation of severe loneliness prevalence in long-term-care residents (up to 42 percent) compared to community-dwelling older adults (10 percent); see *Building Up*, Part One, Section 1.10, for the documented health-and-stability cascade.
22. Ontario long-term care spousal-beds regulation, in force January 1, 2018; Government of Alberta acknowledged capacity need of 10,000 new continuing-care spaces over three years, per the March 2024 funding announcement (Big West Country, “Alberta’s Government Announces Funding for Improved Access to Healthcare,” March 15, 2024).